

Adjuvant abemaciclib + endocrine therapy

Breast - NCCP 00619 - version 4a

Your medicines

Abemaciclib - Endocrine therapy

Your treatment schedule

Abemaciclib twice daily continuously with the prescribed endocrine treatment.

A picture of side effects

Tell your team about new or worsening symptoms. Numbers link each symptom box to the body map.

Abemaciclib - targeted tablets

Endocrine therapy - additional effects

1 Bowel

Abemaciclib

Diarrhoea can occur early. Follow your prescribed plan and contact your team promptly.

3 Liver

Abemaciclib

Liver blood tests are monitored. Report yellow eyes or skin or dark urine.

5 Blood clots

Abemaciclib / treatment context

New one-sided leg swelling, sudden chest pain or breathlessness needs urgent assessment.

2 Blood / infection

Abemaciclib

Blood tests monitor counts. Report fever, shivers or sudden illness immediately.

4 Lungs

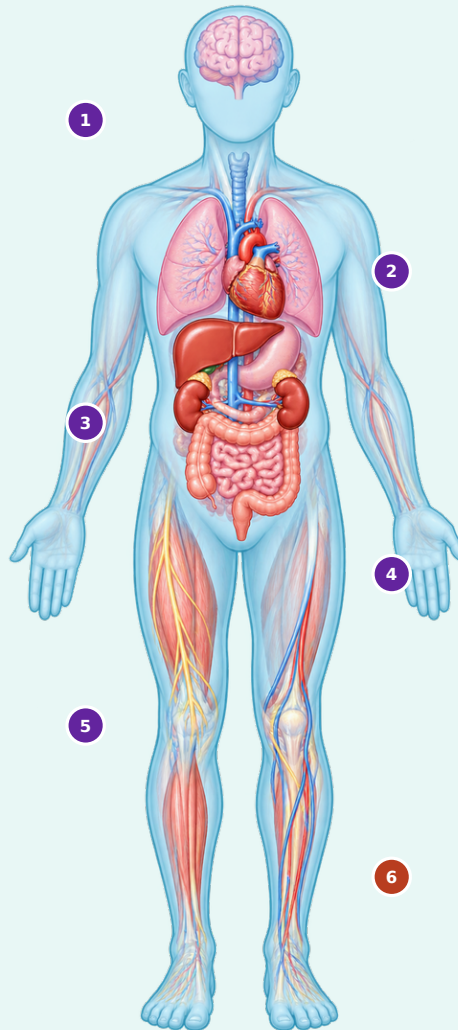
Abemaciclib

Report new cough or breathlessness promptly.

6 Joints / bones

Endocrine therapy

Report troublesome joint symptoms; discuss bone-health monitoring with your team.



Selected effects only. The map does not show likelihood and cannot diagnose the cause of a symptom.

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Your treatment journey

Every day: Abemaciclib 150 mg twice daily, with or without food, at approximately the same times each day unless your dose has been changed.

Alongside it: Continue the endocrine treatment prescribed for you.

First 2 months: Blood count, kidney/liver and bone profile are checked every 2 weeks.

Next 2 months: These tests are usually monthly, then as clinically indicated.

Treatment duration: Abemaciclib is taken continuously for 2 years or until progression/unacceptable toxicity, according to the NCCP source and your treatment plan.

Monitoring and support

- Start anti-diarrhoeal treatment at the first sign of loose stools exactly as advised by your oncology team.
- Swallow tablets whole; do not chew, split or crush.
- Avoid grapefruit/grapefruit juice. If a dose is missed or vomited, take the next dose at the scheduled time rather than an extra dose.
- Your current medicines, including non-prescription products.
- A note of new symptoms, when they started and what they stopped you doing.
- Any temperature, blood-pressure or bowel-count readings your team asked you to record.
- Questions about the purpose of treatment, scan/review timing or changes to the plan.

WHEN TO CALL

Call oncology urgently for fever, persistent or severe diarrhoea, dehydration, new breathlessness, one-sided leg swelling, jaundice, significant bleeding or marked weakness. Severe chest pain or breathing difficulty: call 112/999. If your team has given you different local thresholds or emergency instructions, follow those instructions.

Consent discussion support

Use the approved consent process to document treatment intent, expected benefit, material risks, alternatives, questions and the patient's decision.

Scan to open the regimen-specific patient portal

<https://sactcheck.com/patient/00619/>

Official NCCP 00619 v4a

SACTCheck v0.80.4 - patient information prototype for clinical review. Follow your oncology team and local emergency instructions.

